

Employee Tuberculosis Screening and Respiratory Protection

Policy Number	POL-SAF-074	Owner / Department	Employee Health / Infection Prevention & Control
Effective Date	2023-08-01	Version	2.4
Last Revised	2026-01-22	Next Review	2028-01-22
Approved By	Infection Prevention & Control Committee	Applies To	All employees, licensed providers, students, volunteers, and contractors with potential occupational exposure to <i>Mycobacterium tuberculosis</i>

1. Purpose and Scope

This policy establishes tuberculosis (TB) screening for health-care personnel and the respiratory-protection program for airborne-pathogen exposure. It follows current CDC recommendations for TB screening, testing, and treatment of health-care personnel and the OSHA Respiratory Protection standard, **29 CFR 1910.134**.

2. TB Screening Requirements

All personnel receive a **baseline (preplacement) TB screening**: a symptom and risk assessment plus a TB test (IGRA blood test or two-step tuberculin skin test) unless a prior positive is documented. Routine serial testing of personnel without known exposure is not recommended absent ongoing transmission risk; testing frequency otherwise follows the exposure-risk assessment below.

Risk category	Baseline	After exposure
No ongoing exposure risk	Symptom/risk assessment + TB test	Test on documented exposure only
Potential / recurring exposure	Symptom/risk assessment + TB test	Annual symptom assessment; education
Documented unprotected exposure	Not applicable	Test at baseline and again at 8–10 weeks
Prior positive TB test	Symptom screen; chest imaging as indicated	Annual symptom screen (no repeat test)

3. Respiratory Protection

- Personnel entering airborne-isolation rooms or caring for suspected/confirmed TB patients wear a NIOSH-approved N95 or higher respirator.
- Respirator users are **medically cleared, fit-tested, and trained** before use and fit-tested at least annually and when facial features change.
- Patients with suspected pulmonary TB are placed in an airborne-infection-isolation (negative-pressure) room under Airborne Precautions.
- A user seal check is performed each time a respirator is donned.

IMPORTANT — After an unprotected exposure to a patient with infectious TB, report to Employee Health within 24 hours. You will be screened at baseline and retested at 8–10 weeks; do not wait for symptoms.

4. Severity & Reporting

Classify and notify per **POL-RM-013**. A cluster of occupational TB conversions or confirmed nosocomial transmission is a **SEV-1**; a single confirmed conversion or a significant respiratory-protection failure is a **SEV-2**; an individual unprotected exposure without conversion, or a Near Miss, is a **SEV-3 / Near Miss**.

Severity	Notify	Deadline
SEV-1 (catastrophic / sentinel)	Employee Health, Infection Prevention, administrator on call	Within 1 hour
SEV-2 (major)	Employee Health and Infection Prevention	Within 4 hours
SEV-3 / Near Miss (minor)	Employee Health via electronic event report	Within 24 hours

Bloodborne or sharps aspects of an exposure are cross-referenced to **POL-EH-001**.

5. References

- CDC recommendations for TB screening, testing, and treatment of health-care personnel.
- 29 CFR 1910.134 — OSHA Respiratory Protection standard (medical evaluation, fit testing, training).
- POL-RM-013 — Incident Reporting and Event Severity Classification.
- POL-EH-001 — Bloodborne Pathogen Exposure and Needlestick.